CHAPTER IV
PRESENTATION ANALYSIS AND INTERPRETATION OF DATA

This chapter presents the profile and survey responses of elderly patients and caregivers regarding the existing caregiving process and the proposed ElderCareAI system. The findings are organized by respondent group and questionnaire section, with figures, frequency tables and a discussion for each topic.

RESULTS TEMPLATE — Replace bracketed fields with verified survey results. No response counts, percentages, means, rankings or research findings have been supplied or assumed.

Profile of the respondents

The elderly respondent profile covers age, sex, length of stay at the home of the aged, reported health conditions and comfort with wearing a tracker. The caregiver profile covers age, sex, years of experience, educational attainment, number of patients, familiarity with work-related mobile applications and health indicators that are challenging to monitor manually. These variables provide context for the needs and expectations expressed in the two forms.

Table 1

Respondent groups and usable survey records

Respondent group	Usable responses	Excluded records
Elderly patients — Form A	[N_A]	[n]
Caregivers — Form B	[N_B]	[n]

Record exclusions and their reasons after checking consent, eligibility, duplicates and incomplete responses. Each group has its own denominator; do not assume equal group sizes or that the same people answered both forms.

Presentation sequence: elderly profile; caregiver profile; existing-care ratings; proposed-system ratings; caregiver feature ratings; recommendations; synthesis. Begin with Figure 1 and Table 1 here, then renumber consistently when this chapter is merged into the thesis.

Profile of elderly respondents

Age of Elderly Respondents

Survey item: Age

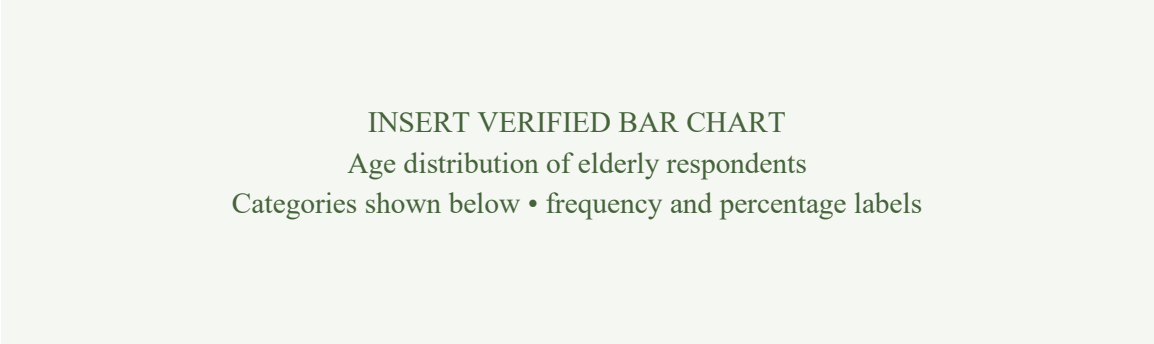


Figure 1: **Age distribution of elderly respondents**

Table 2

Age distribution of elderly respondents

Category	Frequency	Percentage
60-69 years old	[n]	[%]
70–79 years old	[n]	[%]
80 years old and above	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_A] usable elderly records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 1 shows the distribution of elderly respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] The age distribution describes the older adults represented in the survey. It can inform how the team plans readable screens and assisted use, but age alone does not establish a participant's ability to use a wearable.

Sex of Elderly Respondents

Survey item: Sex

INSERT VERIFIED BAR CHART
Sex distribution of elderly respondents
Categories shown below • frequency and percentage labels

Figure 2: **Sex distribution of elderly respondents**

Table 3

Sex distribution of elderly respondents

Category	Frequency	Percentage
Male	[n]	[%]
Female	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_A] usable elderly records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 2 shows the distribution of elderly respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] This distribution describes the composition of the elderly respondent group. It should not be used on its own to claim differences in health status or technology acceptance.

Length of Stay at the Home of the Aged

Survey item: Length of Stay at the Home of the Aged



Figure 3: **Length of stay of elderly respondents**

Table 4

Length of stay of elderly respondents

Category	Frequency	Percentage
Less than 1 year	[n]	[%]
1–3 years	[n]	[%]
4–6 years	[n]	[%]
7 years and above	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_A] usable elderly records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 3 shows the distribution of elderly respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] Length of stay provides context for participants' experience with the facility's existing care arrangements. A longer stay does not itself demonstrate better or poorer care.

Reported Health Conditions

Survey item: Do you currently have any of the following health conditions? (Check all that apply)

INSERT VERIFIED HORIZONTAL BAR CHART
Health conditions reported by elderly respondents
Categories shown below • frequency and percentage labels

Figure 4: **Health conditions reported by elderly respondents**

Table 5

Health conditions reported by elderly respondents

Category	Frequency	Percentage
Heart condition	[n]	[%]
Diabetes	[n]	[%]
Hypertension (High Blood Pressure)	[n]	[%]
Respiratory condition	[n]	[%]
Other (specified)	[n]	[%]

Base: [N_A] usable elderly records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Multiple responses allowed. Percentage = respondents selecting a category ÷ valid respondents answering this item × 100. Percentages can exceed 100% in total; do not divide by the total number of selections. Code specific Other answers before combining them.

The form has no explicit None option for health conditions. Do not interpret an unselected checkbox or missing answer as proof of no condition.

Figure 4 presents the elderly respondents' selections. The most frequently selected category was [category], identified by [n] respondents ([%]), followed by [category] with [n] ([%]). [Describe any ties or other notable selections using the actual counts.] These are self-reported conditions, not diagnoses established by the survey or ElderCareAI. They describe care context and do not establish disease prevalence beyond the participating respondents.

Comfort with Wearing a Wristband or Tracker

Survey item: How comfortable are you with wearing a wristband or tracker?

INSERT VERIFIED HORIZONTAL BAR CHART
Elderly respondents' comfort with wearing a tracker
Categories shown below • frequency and percentage labels

Figure 5: **Elderly respondents' comfort with wearing a tracker**

Table 6

Elderly respondents' comfort with wearing a tracker

Category	Frequency	Percentage
Very Comfortable	[n]	[%]
Comfortable	[n]	[%]
Neutral	[n]	[%]
Uncomfortable	[n]	[%]
Very Uncomfortable	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_A] usable elderly records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 5 shows the distribution of elderly respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] Reported comfort helps identify acceptance concerns that may need explanation, fit adjustments or assisted use. An expression of comfort does not confirm long-term use or clinical benefit.

Profile of caregiver respondents

Age of Caregiver Respondents

Survey item: Age



Figure 6: **Age distribution of caregiver respondents**

Table 7

Age distribution of caregiver respondents

Category	Frequency	Percentage
18–25 years old	[n]	[%]
26–35 years old	[n]	[%]
36–45 years old	[n]	[%]
46 years old and above	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 6 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] The caregiver age distribution provides context for the respondent group. Avoid assuming digital skill from age; the form asks separately about mobile-application familiarity.

Sex of Caregiver Respondents

Survey item: Sex

INSERT VERIFIED BAR CHART
Sex distribution of caregiver respondents
Categories shown below • frequency and percentage labels

Figure 7: **Sex distribution of caregiver respondents**

Table 8

Sex distribution of caregiver respondents

Category	Frequency	Percentage
Male	[n]	[%]
Female	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 7 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] The distribution describes who participated in the caregiver survey. It does not establish differences in monitoring performance or caregiving ability.

Years of Caregiving Experience

Survey item: Years of Experience as a Caregiver

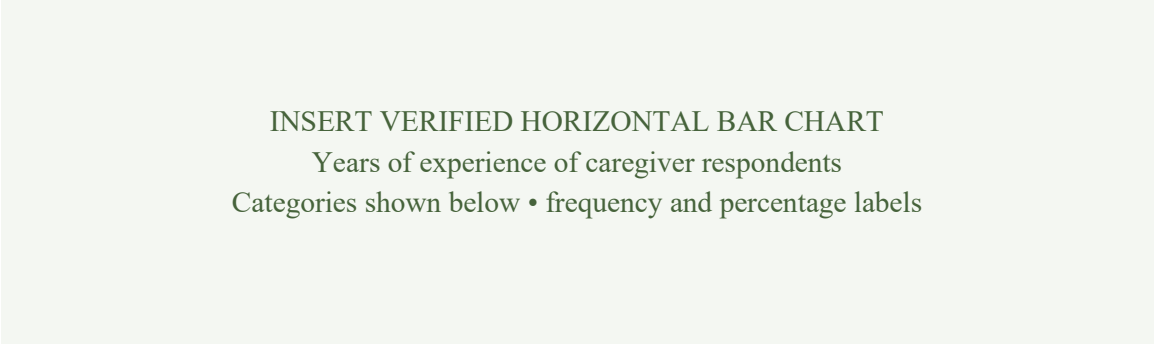


Figure 8: **Years of experience of caregiver respondents**

Table 9

Years of experience of caregiver respondents

Category	Frequency	Percentage
Less than 1 year	[n]	[%]
1–2 years	[n]	[%]
3–5 years	[n]	[%]
6–10 years	[n]	[%]
More than 10 years	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 8 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] Experience describes the respondents' exposure to caregiving work. Interpret it alongside their responses about workload and monitoring practices rather than treating it as proof of expertise.

Highest Educational Attainment

Survey item: Highest Educational Attainment:



Figure 9: **Educational attainment of caregiver respondents**

Table 10

Educational attainment of caregiver respondents

Category	Frequency	Percentage
High School Graduate	[n]	[%]
Vocational/Technical Course	[n]	[%]
College Graduate	[n]	[%]
Other (specified)	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 9 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] Educational attainment describes the group's reported educational background. Because this question is optional, disclose missing responses and use the valid-answer count for percentages.

Number of Elderly Patients Under Care

Survey item: How many elderly patients are currently under your care?



Figure 10: **Patient workload reported by caregiver respondents**

Table 11

Patient workload reported by caregiver respondents

Category	Frequency	Percentage
1–5 patients	[n]	[%]
6–10 patients	[n]	[%]
11–20 patients	[n]	[%]
More than 20 patients	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 10 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] The patient-count categories describe reported workload. They can contextualize responses about simultaneous monitoring, but the survey does not establish that workload caused a specific problem.

Familiarity with Work Related Mobile Applications

Survey item: How familiar are you with using mobile applications for work-related tasks?

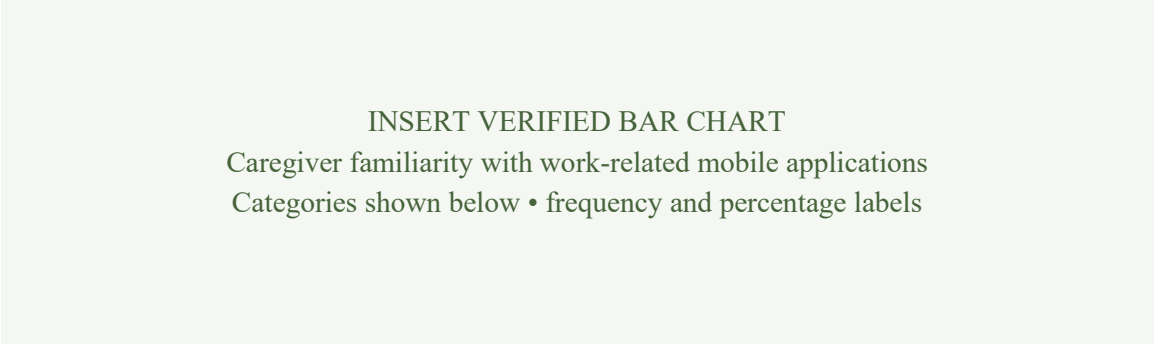


Figure 11: **Caregiver familiarity with work-related mobile applications**

Table 12

Caregiver familiarity with work-related mobile applications

Category	Frequency	Percentage
Very Familiar	[n]	[%]
Familiar	[n]	[%]
Unfamiliar	[n]	[%]
Very Unfamiliar	[n]	[%]
Total valid answers	[n_valid]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Percentage = category frequency ÷ valid answers to this item × 100. Keep the category order shown in the questionnaire and label the frequency axis from zero.

Figure 11 shows the distribution of caregiver respondents for this item. Of [n_valid] valid answers, [n] ([%]) fell under [category], while [n] ([%]) were in [category]. [State the largest group only after checking for ties.] The responses describe familiarity with work-related apps. Use the actual four options in the form; no Neutral option was provided for this question.

Health Indicators Challenging to Monitor Manually

Survey item: Which health indicators do you find most challenging to monitor manually? (Check all that apply)



Figure 12: **Health indicators caregivers find challenging to monitor manually**

Table 13

Health indicators caregivers find challenging to monitor manually

Category	Frequency	Percentage
Heart Rate	[n]	[%]
Blood Oxygen Level (SpO2)	[n]	[%]
Sleep Pattern	[n]	[%]
Stress Level	[n]	[%]
Skin Temperature	[n]	[%]
Physical Activity / Fitness	[n]	[%]
Other (specified)	[n]	[%]

Base: [N_B] usable caregiver records; this item has [n_valid] valid answers and [n_missing] unanswered records.

Multiple responses allowed. Percentage = respondents selecting a category ÷ valid respondents answering this item × 100. Percentages can exceed 100% in total; do not divide by the total number of selections. Code specific Other answers before combining them.

Figure 12 presents the caregiver respondents' selections. The most frequently selected category was [category], identified by [n] respondents ([%]), followed by [category] with [n] ([%]). [Describe any ties or other notable selections using the actual counts.] Selections identify areas of reported difficulty and can inform feature priorities. They do not establish that ElderCareAI measures each indicator continuously or with medical accuracy.

Ratings of the current caregiving process and proposed system

Present the response distributions for each statement separately for elderly patients and caregivers. The tables below retain the statements as administered. They describe respondents' opinions and expectations, rather than measured changes in health outcomes or proven system performance.

Table 14

Coding for the five point agreement scale

Response	Score
Strongly Agree (SA)	5
Agree (A)	4
Neutral (N)	3
Disagree (D)	2
Strongly Disagree (SD)	1

For each statement, report the five response counts, the number of valid answers and the item mean. Item mean = (5 × SA + 4 × A + 3 × N + 2 × D + 1 × SD) ÷ valid answers to that statement. Retain blank or missing answers separately; do not code them as Neutral or zero.

Optional interpretation bands for this template: 4.21–5.00 Strongly Agree; 3.41–4.20 Agree; 2.61–3.40 Neutral; 1.81–2.60 Disagree; 1.00–1.80 Strongly Disagree. Use these only if they match the approved methodology. Classify the unrounded mean and then display it to two decimals.

Direction of the statements

Elderly negative statements: A-B5 (delays), A-B7 (monitoring problems) and A-C5 (concern about all-day wear). Caregiver negative statements: B-B2 (tracking difficulty), B-B5 (time-consuming monitoring), B-B6 (overwhelmed) and B-B7 (process problems). Higher raw scores indicate stronger agreement with those concerns.

Do not average mixed-direction items and call the result satisfaction or effectiveness. If a composite is justified by the methodology and scale validation, reverse the negative items using 6 − original score, define the construct, and disclose the recoding. Otherwise report the item distributions without a composite.

Elderly Ratings of the Current Monitoring Process

Source: Form A — elderly patients. Report each item using its own valid-response count. SA/A/N/D/SD are counts; M is the item mean; VI is the verbal interpretation.

Table 15

Elderly Ratings of the Current Monitoring Process

Item	Statement	SA	A	N	D	SD	M	VI
A-B1	My health vitals (e.g., heart rate, blood pressure, temperature) are regularly checked by my caregiver.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B2	I feel that my health condition is closely monitored at all times.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B3	I am promptly informed when there are changes in my health condition.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B4	I feel safe and secure under the current care setup in this facility.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B5	I experience delays in receiving medical attention when I need it urgently. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B6	The current method of monitoring my health is sufficient for my needs.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-B7	There are problems I encounter with how my health is currently being monitored. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]

For every row: valid n = SA + A + N + D + SD. If item denominators differ, state the valid n beside its result or in a table note. [negative] is an analysis marker, not part of the original survey wording.

Table 15 presents the responses to these statements. The highest raw item mean was [M] for [item], interpreted as [VI], while [item] had [M]. [Describe the observed response distribution and any ties.] Interpret the statement's wording before discussing agreement; agreement with a negative statement indicates a reported concern.

Discussion focus: reported regularity of checks, perceived monitoring coverage, communication of health changes, safety, delays, adequacy and problems. The existing-care question mentions blood pressure; this is not a claim that ElderCareAI currently measures blood pressure.

Caregiver Ratings of the Current Caregiving Process

Source: Form B — caregivers. Report each item using its own valid-response count. SA/A/N/D/SD are counts; M is the item mean; VI is the verbal interpretation.

Table 16

Caregiver Ratings of the Current Caregiving Process

Item	Statement	SA	A	N	D	SD	M	VI
B-B1	I am able to effectively monitor the health vitals of all elderly patients under my care.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B2	I face difficulties in tracking the health status of multiple elderly patients simultaneously. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B3	I am promptly informed when an elderly patient experiences a health emergency.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B4	The current tools and methods available for monitoring elderly patients are sufficient.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B5	Manually monitoring the health of elderly patients is time-consuming. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B6	I often feel overwhelmed by the number of patients I need to monitor at the same time. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B7	There are significant problems encountered in the current caregiving and monitoring process. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-B8	The existing process of documenting and reporting patient health conditions is efficient.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]

For every row: valid n = SA + A + N + D + SD. If item denominators differ, state the valid n beside its result or in a table note. [negative] is an analysis marker, not part of the original survey wording.

Table 16 presents the responses to these statements. The highest raw item mean was [M] for [item], interpreted as [VI], while [item] had [M]. [Describe the observed response distribution and any ties.] Interpret the statement's wording before discussing agreement; agreement with a negative statement indicates a reported concern.

Discussion focus: coverage of multiple patients, emergency communication, adequacy of existing tools, time burden, workload and documentation. Do not describe these baseline opinions as a post-deployment evaluation.

Elderly Perceptions of a Health Monitoring Wearable

Source: Form A — elderly patients. Report each item using its own valid-response count. SA/A/N/D/SD are counts; M is the item mean; VI is the verbal interpretation.

Table 17

Elderly Perceptions of a Health Monitoring Wearable

Item	Statement	SA	A	N	D	SD	M	VI
A-C1	I would be willing to wear a smartwatch that continuously monitors my health.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-C2	I believe a wearable health device would help my caregiver monitor my condition more effectively.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-C3	I would feel more secure knowing that my caregiver is notified in real time whenever my health shows abnormal readings.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-C4	I believe technology such as a health-monitoring smartwatch can improve my overall safety and well-being.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-C5	I am concerned about wearing a health-monitoring device throughout the day. [negative]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
A-C6	I think an automatic alert system to caregivers during health emergencies would be beneficial.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]

For every row: valid n = SA + A + N + D + SD. If item denominators differ, state the valid n beside its result or in a table note. [negative] is an analysis marker, not part of the original survey wording.

Table 17 presents the responses to these statements. The highest raw item mean was [M] for [item], interpreted as [VI], while [item] had [M]. [Describe the observed response distribution and any ties.] Interpret the statement's wording before discussing agreement; agreement with a negative statement indicates a reported concern.

Discussion focus: willingness to wear a smartwatch, expected support and safety, alert expectations and concern about prolonged wear. A-C5 measures concern; it must not be counted as positive acceptance without an explicitly justified reversal.

Caregiver Perceptions of the Proposed System

Source: Form B — caregivers. Report each item using its own valid-response count. SA/A/N/D/SD are counts; M is the item mean; VI is the verbal interpretation.

Table 18

Caregiver Perceptions of the Proposed System

Item	Statement	SA	A	N	D	SD	M	VI
B-C1	I believe that integrating a smartwatch for health monitoring will improve patient care.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C2	Using AI to analyze elderly patients' vital signs will help detect health risks more accurately and quickly.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C3	AI-driven health risk prediction and anomaly detection will allow for faster caregiver response during emergencies.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C4	Automated notifications sent to caregivers when abnormal vitals are detected will be effective in preventing health complications.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C5	An AI chatbot that provides guidance and support to caregivers will be a helpful tool in daily caregiving tasks.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C6	AI-generated recommendations for caregivers based on patient health data will support better decision-making.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-C7	Monitoring of heart rate, blood oxygen, sleep, stress, skin temperature, and fitness activity will give a more complete picture of a patient's health.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]

For every row: valid n = SA + A + N + D + SD. If item denominators differ, state the valid n beside its result or in a table note. [negative] is an analysis marker, not part of the original survey wording.

Table 18 presents the responses to these statements. The highest raw item mean was [M] for [item], interpreted as [VI], while [item] had [M]. [Describe the observed response distribution and any ties.] Interpret the statement's wording before discussing agreement; agreement with a negative statement indicates a reported concern.

Discussion focus: expected usefulness of smartwatch monitoring, AI guidance, predictions, notifications and more complete records. Agreement expresses expectations; it does not prove improved accuracy, prevented complications or faster emergency response.

Caregiver Ratings of Proposed Key Features

Source: Form B — caregivers. Report each item using its own valid-response count. SA/A/N/D/SD are counts; M is the item mean; VI is the verbal interpretation.

Table 19

Caregiver Ratings of Proposed Key Features

Item	Statement	SA	A	N	D	SD	M	VI
B-D1	A heart rate monitoring feature on the mobile app will help me detect cardiac irregularities in elderly patients promptly.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D2	The sleep tracking feature will give me valuable insights into patients' rest quality and help me adjust care plans accordingly.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D3	The SpO_2 tracking feature will reduce the likelihood of overlooking dangerous drops in blood oxygen levels.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D4	Having stress level data from the smartwatch will enable me to provide more holistic and timely care.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D5	The skin temperature deviation feature will be useful for early identification of fever or other temperature-related health conditions.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D6	The fitness activity tracking feature will help detect sudden changes in mobility that may signal a health concern.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D7	The AI-powered vital signs analysis feature will provide more accurate health insights compared to manual observation.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D8	AI-based health risk prediction will enable me to take preventive actions before a patient's condition worsens.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D9	The anomaly detection feature will effectively flag unusual health patterns that require immediate caregiver attention.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D10	Automated caregiver notifications when abnormal vitals are detected will significantly improve my emergency response time.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D11	The AI chatbot will help me save time during the day by providing quick summaries of the patient's status without me having to check multiple devices.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]
B-D12	The automated health report generation feature will reduce the time I spend on manual documentation of patient health data.	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]	[ ]

For every row: valid n = SA + A + N + D + SD. If item denominators differ, state the valid n beside its result or in a table note. [negative] is an analysis marker, not part of the original survey wording.

Table 19 presents the responses to these statements. The highest raw item mean was [M] for [item], interpreted as [VI], while [item] had [M]. [Describe the observed response distribution and any ties.] Interpret the statement's wording before discussing agreement; agreement with a negative statement indicates a reported concern.

Discussion focus: heart rate, sleep, SpO2, stress, wrist temperature, activity, AI insights, predictions, anomaly flags, notifications, Elle summaries and reports. Keep the original statements in the results table even where their proposed capabilities exceed what has been clinically demonstrated.

Respondent recommendations

Both forms include an optional recommendations field. Summarize the submitted suggestions by theme, keeping elderly and caregiver responses identifiable as separate groups. Omit names, health identifiers and recognizable personal details from quotations. Do not treat a blank field as a recommendation or as satisfaction.

Table 20

Themes from optional respondent recommendations

Respondent group	Theme	Respondents mentioning theme	Anonymous supporting excerpt
Elderly patients	[theme]	[n]	[verified excerpt]
Caregivers	[theme]	[n]	[verified excerpt]

Count distinct respondents mentioning a theme, rather than repeated words in the same response. A response can contain several themes, so theme counts may overlap. If no recommendations were submitted, report zero submitted comments and omit unsupported themes.

Among [n] submitted recommendation responses from elderly patients and [n] from caregivers, the recurring themes were [themes]. [Describe only themes supported by the actual comments.] These suggestions can inform practical improvements, but they do not substitute for technical tests or professional clinical assessment.

Synthesis of survey findings

The elderly profile indicates [supported profile finding], while the caregiver profile indicates [supported profile finding]. The current-process ratings identify [documented strength or concern]. Responses about the proposed system indicate [supported expectation or concern]. The recommendations emphasize [verified themes]. These findings provide context for system design and further evaluation; they do not demonstrate clinical effectiveness.

Keep the two respondent groups separate. Their age categories, roles and most statements differ. Do not report a pooled overall mean or present unmatched statements as a direct statistical comparison.

Reporting notes and questionnaire sources

Table 21

What to present from the two forms

Data to show	Recommended presentation
Elderly profile — 5 items	Figures 1–5 and frequency/percentage tables
Caregiver profile — 7 items	Figures 6–12 and frequency/percentage tables
Existing process — 7 elderly and 8 caregiver items	Separate five-category distributions and item means
Proposed system — 6 elderly and 7 caregiver items	Separate agreement tables with direction notes
Proposed key features — 12 caregiver items	Caregiver-only agreement table
Optional recommendations — both forms	Anonymous themes and respondent counts

No city, address, income, religion, patient diagnosis confirmation, measured response time, SUS score or ISO quality score is collected in these two forms. Do not add such findings without another identified data source. Names and contact details should not appear in respondent-profile charts.

Minor display cleanup: the caregiver experience option “) 6–10 years” is shown as “6–10 years”; “Strongly DIsagree” is standardized to “Strongly Disagree.” These changes do not change the category or response coding. Preserve meaningful Other responses and document their grouping.

Clinical interpretation: wrist skin temperature is not core temperature or a fever diagnosis; the HRV-derived stress index is experimental; recorded sleep is imported after synchronization. Responses about cardiac irregularities, prediction accuracy or prevention of complications are perceptions and must not be presented as validated medical outcomes.

Replace each interpretation prompt only after calculating the results. Add verified relevant literature if required by the thesis format; do not reuse the energy-monitoring references or numerical findings from the sample PDF.

Survey Form A for elderly patients

Survey Form B for caregivers

Format reference: 228.pdf supplied by the researchers. Form structure checked on 10 November 2026. All bracketed results remain blank for the actual response exports.